GOVCARE EHR

CKD AI CLINICAL DECISION SUPPORT REPORT

Clinical decision support only - clinician review required

PATIENT & REPORT DETAILS

Patient	Operational software QA; not a clinical subject
Patient number	OPERATIONAL-FEATURE-QA
DOB	Not recorded
Sex	Not recorded
Assessment ID	237255db-f629-4e5b-96c3-7e592cb706d6
Assessment time	Not recorded
Hospital	Not recorded
Department	Not recorded
Clinician / operator	Not recorded
Model	Software verification only

CKD RISK SUMMARY

Predicted CKD risk	Risk category	AI confidence
N/A	**N/A**	**N/A**

CKD CLASSIFICATION & CLINICAL CODING

Suggested CKD classification	Unable to classify
G category	Not assigned
Suggested ICD code	Not assigned
Clinical description	Not assigned
Terminology system	Not assigned
Basis	Reviewed eGFR unavailable
Coding status	AI-assisted suggestion
Confirmation	Clinician confirmation required

A single eGFR result does not independently establish chronic kidney disease or chronicity. N18.6 (ESRD) is not inferred from eGFR alone.

○ G1	○ G2	○ G3a	○ G3b	○ G4	○ G5

RENAL & ASSOCIATED FINDINGS

Parameter	Result	Unit	Source
eGFR	35.0	mL/min/1.73m²	MANUAL
Serum creatinine	2.0	mg/dL	MANUAL
Urine albumin/creatinine ratio (ACR)	420.0	mg/g	MANUAL
Urine protein	2.0	mg/dL	MANUAL
Systolic blood pressure	148.0	mmHg	MANUAL

Diastolic blood pressure	92.0	mmHg	MANUAL
HbA1c	7.1	%	MANUAL
Serum potassium	5.8	mmol/L	MANUAL
Hemoglobin	9.8	g/dL	MANUAL

CKD Related Clinical Intelligence Summary

Source evidence, monitoring status and next clinical actions

Assessment-linked snapshot: d303ab9c-79d1-45b7-ad66-9e3708f2259e | Evaluated: 2026-10-14T03:46:46.341663531Z

Anemia Monitoring

Risk status: Clinical review recommended

Current Hb: 9.8 g/dL | 2026-10-13

Clinical Data Readiness: 100% Ready | Available evidence: 1/1 monitoring inputs

Clinical indicators: Hemoglobin 9.8 g/dL

Missing information: Listed monitoring inputs recorded

Extended workup: PCV, RBC count, ferritin and iron profile are not linked to this snapshot; review CBC / iron-study sources.

Dated trend: 11.3 to 9.8 g/dL

AI explanation / clinical rule: Hemoglobin 9.80 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.

Research prediction: Eligible dialysis context not verified

Clinical action: Add historical Hb result

Clinician notes: No notes stored for this module snapshot.

Hyperkalemia Risk Monitoring

Risk status: Clinical review recommended

Current potassium: 5.8 mmol/L | 2026-10-13

Clinical Data Readiness: 100% Ready | Available evidence: 1/1 monitoring inputs

Clinical indicators: Serum Potassium 5.8 mmol/L

Missing information: Listed monitoring inputs recorded

Dated trend: 5.8 to 5.8 mmol/L

AI explanation / clinical rule: Serum potassium 5.80 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.

Research prediction: Eligible dialysis context not verified

Clinical action: Add historical potassium result

Clinician notes: No notes stored for this module snapshot.

Dialysis Effectiveness Monitoring

Risk status: Data required

URR: Not calculated

Clinical Data Readiness: 0% Ready | Available evidence: 0/3 monitoring inputs

Clinical indicators: Source measurements required

Missing information: Pre-HD Urea; Post-HD Urea; Dialysis date

AI explanation / clinical rule: Insufficient source evidence for interpretation.

URR = ((Pre-HD urea - Post-HD urea) / Pre-HD urea) x 100

Research prediction: Eligible dialysis context not verified

Clinical action: Add dialysis-session data

Clinician notes: No notes stored for this module snapshot.

Renal Function Progression

Risk status: Monitoring

Measured creatinine change: 25% (not probability)

Clinical Data Readiness: 100% Ready | Available evidence: 4/4 monitoring inputs

Clinical indicators: Serum Creatinine 2 mg/dL

Missing information: Listed monitoring inputs recorded

Dated trend: 1.6 to 2 mg/dL

AI explanation / clinical rule: Creatinine 1.60 to 2.00 mg/dL over 30.00 days; 25.00% change (not probability).

Clinical action: Add baseline creatinine history

Clinician notes: No notes stored for this module snapshot.

CKD Cardiovascular Monitoring

Risk status: Surveillance recommended | Risk not estimated

Clinical Data Readiness: 80% Ready | Available evidence: 4/5 monitoring inputs

Clinical indicators: Systolic Blood Pressure 148 mmHg; Diastolic Blood Pressure 92 mmHg; Serum Potassium 5.8 mmol/L; Serum Creatinine 2 mg/dL; Age 56 years

Missing information: Clinical diabetes status

AI explanation / clinical rule: The dashboard presents actual blood pressure, potassium, creatinine and age when available, with their source dates.

Prediction readiness: cardiovascular model unavailable; factor completeness is not a risk probability.

Clinical action: Review cardiovascular factors

Clinician notes: Review / record findings in the existing patient clinical record.

Why this result?

Interpret recorded creatinine, eGFR, potassium and hemoglobin with their actual source dates. Arrows and trends require repeated dated observations. Missing values do not imply low risk.
Clinical monitoring uses transparent existing measurement rules. Research forecasts remain separately gated. Detailed source evidence, model information, limitations and stored clinician reviews follow.

Data completeness is not diagnostic certainty, forecast eligibility or disease probability. No inference is run during export.

CKD-RELATED CLINICAL RISKS & PREDICTIVE INTELLIGENCE

Sri Lankan Longitudinal Renal & Hemodialysis Data

Saved snapshot: d303ab9c-79d1-45b7-ad66-9e3708f2259e | Evaluated 2026-10-14T03:46:46.341663531Z. Export reads saved clinical evidence; no inference is performed.

Clinical Decision Support / Research Use. Outputs require clinician interpretation. Research-model outputs are not independently validated diagnoses.

Prediction generated from available clinical measurements. Longitudinal forecasting requires repeated dated observations. Clinical data readiness is input completeness, not diagnostic certainty or disease probability.

SIX-MODULE EVIDENCE & CLINICIAN REVIEW

CKD Progression / Renal Trajectory

CALCULATED - High CKD progression monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Transparent snapshot monitoring rule

Qualitative measurement-based monitoring band; no future-event probability or forecast horizon is estimated.

Previous dated observations available; trends are shown separately from snapshot risk.

Clinical Data Readiness: 89% Ready | 8/9 parameters | Missing: 1

GovCare qualitative eGFR/ACR grid: High if eGFR <45, ACR >300, or eGFR <60 with ACR ≥30; Moderate if eGFR <60 or ACR ≥30; Low requires eGFR ≥60 AND ACR <30. Categories condense high/very-high monitoring; not a validated progression model.

Serum Creatinine: 2 mg/dL | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

eGFR: 35 mL/min/1.73m² | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Protein in urine: 2 mg/dL | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Urine ACR: 420 mg/g | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Age: 56 years | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Systolic Blood Pressure: 148 mmHg | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Diastolic Blood Pressure: 92 mmHg | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

HbA1c: 7.1 % | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Clinical recommendations

Clinician review of eGFR, ACR, urine findings and renal chronicity; obtain previous dated observations for longitudinal forecasting.

Why: Reduced eGFR: 35.00 mL/min/1.73 m2.

Why: Elevated ACR: 420.00 mg/g.

Why: Serum creatinine: 2.00 mg/dL; interpret against the patient's source reference range.

Why: HbA1c / glucose are supporting glycemic measurements; a diabetes diagnosis is not inferred.

Why: Serum Creatinine increased: 1.60 to 2.00 mg/dL across the recorded source dates.

Why: eGFR decreased: 45.00 to 35.00 mL/min/1.73m2 across the recorded source dates.

Why: Urine ACR unchanged: 420.00 to 420.00 mg/g across the recorded source dates.

Available: Serum Creatinine | 1/1 | 2.00 mg/dL; source date recorded

Available: eGFR | 1/1 | 35.00 mL/min/1.73m²; source date recorded

Available: Protein in urine | 1/1 | 2.00 mg/dL; source date recorded

Available: Urine ACR | 1/1 | 420.00 mg/g; source date recorded

Available: Age | 1/1 | 56.00 years; source date recorded

Available: Systolic Blood Pressure | 1/1 | 148.00 mmHg; source date recorded

Available: Diastolic Blood Pressure | 1/1 | 92.00 mmHg; source date recorded

Needs review / data: Fasting Blood Sugar | 0/1 | No recorded value

Available: HbA1c | 1/1 | 7.10 %; source date recorded

Next action: Add historical laboratory data

Directions compare the two latest valid values per domain using a 1% neutral band; this is not a prognosis or a G1-to-G5 forecast.

Dialysis timing, clinical context and measurement uncertainty require clinician interpretation.

Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Kidney Failure / G5D

CALCULATED - Kidney failure risk indicator calculated

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Clinical evidence / monitoring rule

High monitoring signal requires eGFR <15 or verified chronic dialysis; creatinine alone cannot establish G5D.

Dialysis verification required

Clinical Data Readiness: 80% Ready | 4/5 parameters | Missing: 1

eGFR: 35 mL/min/1.73m² | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Serum Creatinine: 2 mg/dL | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Protein in urine: 2 mg/dL | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Urine ACR: 420 mg/g | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Clinical recommendations

Review renal function and chronicity; verify dialysis status before G5D labeling or maintenance-HD model use.

Why: eGFR 35.00 mL/min/1.73 m2 does not meet the <15 high-signal threshold.

Available: eGFR | 1/1 | 35.00 mL/min/1.73m²; source date recorded

Available: Serum Creatinine | 1/1 | 2.00 mg/dL; source date recorded

Available: Protein in urine | 1/1 | 2.00 mg/dL; source date recorded

Available: Urine ACR | 1/1 | 420.00 mg/g; source date recorded

Needs review / data: Verified dialysis status | 0/1 | Dialysis verification required

Next action: Verify dialysis status

High creatinine alone does not establish G5D. This card reports recorded clinical evidence, not a diagnosis generated by AI.

No 2-year or 5-year kidney-failure probability is calculated.

Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Hyperkalemia Monitoring

CALCULATED - High potassium monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current potassium monitoring rule

Current measurement monitoring is independent of the maintenance-HD research forecast. The low band does not exclude hypokalemia or other abnormalities.

Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0

Serum Potassium: 5.8 mmol/L | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Clinical recommendations

Review measured potassium with the clinician, source quality, medications and sampling context; use true paired pre/post-HD samples when available.

Why: Serum potassium 5.80 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.

Available: Serum Potassium | 1/1 | 5.80 mmol/L; source date recorded

Next action: Add historical potassium result

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.

Dataset permission status remains UNVERIFIED.

This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.

Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.

Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Anemia / Hb Deterioration

CALCULATED - Anemia monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current hemoglobin monitoring rule

Measured Hb monitoring and decline rate are separate from the installed persistence baseline; the baseline is not an anemia prognosis.

Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0

Hemoglobin: 9.8 g/dL | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Clinical recommendations

Clinician review against age/sex-specific laboratory ranges, iron studies and clinical context; add a previous dated Hb result for deterioration monitoring.

Why: Hemoglobin 9.80 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.

Why: Hb change -1.50 g/dL over 30.00 days; decline rate is positive for a fall and negative for a rise.

Available: Hemoglobin | 1/1 | 9.80 g/dL; source date recorded

Next action: Add historical Hb result

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.

Dataset permission status remains UNVERIFIED.

Learned candidates failed the gate against persistence. Carries forward the last measured Hb; no anemia-classifier probability is installed.

This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.

Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.

Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Dialysis Adequacy / URR

INSUFFICIENT DATA - Dialysis session measurements required

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Measured same-session urea reduction ratio

Requires hemodialysis session data

Clinical Data Readiness: 0% Ready | 0/3 parameters | Missing: 3

URR = ((Pre-HD Urea − Post-HD Urea) / Pre-HD Urea) × 100

Clinical recommendations

Review measured adequacy with the clinician; record actual pre-HD urea, post-HD urea and dialysis date. BUN from the CKD form is not a paired dialysis urea measurement.

Needs review / data: Pre-HD Urea | 0/1 | Requires hemodialysis session data

Needs review / data: Post-HD Urea | 0/1 | Requires hemodialysis session data

Needs review / data: Dialysis date | 0/1 | Actual dialysis session date; upload time is not substituted

Next action: Add dialysis-session data

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.

Dataset permission status remains UNVERIFIED.

This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.

Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.

Clinical feature sources: 0 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

AKI / Renal Clinical Deterioration

CALCULATED - Creatinine change calculated

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Dated creatinine change / acute review rule

Creatinine change is a measured trend. It does not establish AKI; timing, urine output and clinical context require review.

Clinical Data Readiness: 100% Ready | 4/4 parameters | Missing: 0

Creatinine-based clinical review only: rise ≥0.3 mg/dL within 48 h or ≥1.5 times a dated baseline within 7 days. No AKI staging, urine-output assessment or diagnosis is generated.

Creatinine change (%) = ((Current Creatinine − Previous Creatinine) / Previous Creatinine) × 100

Serum Creatinine: 2 mg/dL | 2026-10-13T03:46:46.333119264Z | Source: CKD_ASSESSMENT / 237255db-f629-4e5b-96c3-7e592cb706d6

Clinical recommendations

Obtain previous creatinine with its actual measurement date; clinician review of timing, acute illness, medications and urine output is required.

Why: Creatinine 1.60 to 2.00 mg/dL over 30.00 days; 25.00% change (not probability).
Available: Serum Creatinine | 1/1 | 2.00 mg/dL; source date recorded
Available: Previous creatinine | 1/1 | Baseline creatinine required for AKI trend calculation
Available: Current creatinine measurement date | 1/1 | Current and previous results need distinct actual dates
Available: Previous creatinine measurement date | 1/1 | A recorded baseline date is required
Next action: Add baseline creatinine history
Directional composite – not a validated prognosis score.
Equal weights; two latest valid observations per domain; 1% neutral stability band. Missing domains are excluded, never scored as zero.
A directional percentage is not a disease probability.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

CLINICAL DATA USED FOR ASSESSMENT

Parameter	Result	Unit	Source	Coding
Serum creatinine	2.0	mg/dL	MANUAL	-
eGFR	35.0	mL/min/1.73m²	MANUAL	-
Urine albumin/creatinine ratio (ACR)	420.0	mg/g	MANUAL	-
Serum potassium	5.8	mmol/L	MANUAL	-
Hemoglobin	9.8	g/dL	MANUAL	-
HbA1c	7.1	%	MANUAL	-
Urine protein	2.0	mg/dL	MANUAL	-
Age	56.0	years	MANUAL	-
Systolic blood pressure	148.0	mmHg	MANUAL	-
Diastolic blood pressure	92.0	mmHg	MANUAL	-

KEY FACTORS IN THIS MODEL OUTPUT

Contribution direction describes this model's prediction, not whether a clinical value is healthy, safe or protective.

Factor	Value	Unit	Direction	Contribution

RECOMMENDED CLINICAL REVIEW

No recommendation was returned by the prediction service.

CLINICIAN REVIEW

Suggested classification	Not assigned
Suggested code	Not assigned
Coding decision	[ ] Confirm [ ] Modify [ ] Reject
Final clinician-selected code	_____
Clinical notes	_____ _____
Reviewed by / date	_____

This output does not diagnose disease or replace professional medical judgement. AI-assisted coding must be reviewed before a confirmed FHIR Condition is created.